SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Lomustine + bevacizumab 7.5 mg/kg

NCCP 00804: recurrent malignant glioblastoma; bevacizumab use is off-label. · NCCP 00804 · version 2 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Your team discusses likely benefits, risks, symptoms and quality-of-life goals.

Schedule
Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1 and 22. Up to six cycles are planned, with review before each treatment.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Lomustine
ORAL CHEMOTHERAPY CAPSULE

- ☐ Delayed and cumulative low blood counts
- ☐ Nausea / vomiting
- ☐ Lung injury
- ☐ Kidney and liver changes
- ☐ Rare second cancers and fertility effects

Bevacizumab
TARGETED ANTIBODY GIVEN INTO A VEIN

- ☐ High blood pressure and rare PRES neurological syndrome
- ☐ Protein in the urine and kidney injury
- ☐ Bleeding, including serious internal bleeding
- ☐ Blood clots, stroke or heart attack
- ☐ Slow wound healing and wound complications
- ☐ Gastrointestinal perforation or fistula
- ☐ Infusion reactions and rare severe allergy
- ☐ Fertility effects and harm to a pregnancy

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. Keep steroid and seizure plans separate from anticancer treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00804 v2.

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